

Proposal for Return to Work

Accompanying the Response to the Request for Medical Information · MSH-INJ-5795

Cory Lea Shepherd

AO3, Switchboard Services, Logan Hospital, Metro South Health

To: Injury Management, Human Resources, Logan Bayside Health Network

lbh_injurymanagement@health.qld.gov.au

Cc: LBH.HRTeam1@health.qld.gov.au

Date: 3 August 2026

1. WHAT THIS DOCUMENT IS

This document sets out my proposal for returning to work. It accompanies my response of today's date to the Request for Medical Information, and is presented separately so that it can be considered — and, internally, routed — on its own. The confirmations I seek are listed at Part 9 of the response. The proposal at section 3 below is also the subject of my separate request of today's date under clause 10.3.2 of the certified agreement.

2. THE PROPOSAL

I have been held out of the workplace since 3 July 2026 without wages, with my accrued leave debited, and self-funding the medical costs of this process. G03 clause 1.1 contemplates temporary adjustments which allow an employee to "continue work during their recovery," and clause 1.1.1 confirms that workplace rehabilitation support is available to employees "who have an injury/illness impacting their current ability to perform their role, with no claim or entitlement to compensation (health management case)." Clause 1 provides that "Managers are responsible for ensuring work environments and work, do not unreasonably prevent employment for people."

The Health Service's Workplace rehabilitation standard (QH-IMP-401-5, 7 January 2026) is also engaged. It provides that the line manager or delegate shall "identify and actively seek ... meaningful suitable duties, and ensure suitable duties are made available wherever reasonably practicable"; that the line manager "is best placed to be the primary contact for the worker wherever possible and facilitate the injured workers on-going connection with the workplace"; that "regular, transparent and timely communication" be maintained with the worker "whether at work or off work"; and that the worker be informed of "the appropriate dispute resolution process to follow in such cases where the worker does not agree with elements of a proposed workplace rehabilitation plan or suitable duties program". The standard applies to a worker requiring rehabilitation "for either a work or non-work-related injury/illness".

I have been offered no suitable duties — interim duties were declined in any capacity. I have been held away from the workplace rather than connected to it. I have not been informed of any dispute resolution process. I ask that each of those be addressed.

I also record that I was not consulted before the decision of 3 July 2026, or since. Section 47 of the Work Health and Safety Act 2011 requires consultation with workers who are "directly affected by a matter relating to work health or safety". Section 48 requires that I be given a reasonable opportunity "to contribute to the decision-making process relating to the matter". Section 49 requires consultation when identifying hazards and assessing risks, when deciding how to eliminate or minimise them, when proposing changes that may affect the health or safety of workers, and when making decisions about the procedures for monitoring the health of workers. The present request is a procedure for monitoring my health.

I propose the following. Each item is within the Health Service's existing processes and, so far as I can see, requires no material expenditure and no restructuring.

- (a) RETURN TO DUTY. That I return to my rostered shifts now, on the restrictions certified in the Employee Capabilities Checklist of 3 July 2026, while the balance of this inquiry is completed. If the exclusion is maintained, that the instrument relied on be identified, that the period be treated as paid special leave, and that the leave debited since 3 July 2026 be reversed.
- (b) COMPLAINT HANDLING. That the Health Service confirm in writing that complaints received by Switchboard staff are escalated in accordance with the process it has itself described. This requires no change to anything.
- (c) ROSTER PREDICTABILITY. That my roster be published in accordance with the certified agreement and not altered without my agreement other than in an emergency.

- (d) ORDINARY PROCEDURAL FAIRNESS. That any concern about my performance or conduct be raised with me in writing, with particulars, in accordance with the applicable directive, and that I be afforded a support person.
- (e) A SEPARATE CONTACT POINT. That leave, injury management and human resources matters concerning me be handled by an officer outside Switchboard Services rather than by my line manager, who wrote to me on 2 July 2026 concerning leave types and my return to duty. This does not alter day-to-day supervision and involves no change to reporting lines.
- (f) WORKPLACE RISK ASSESSMENT. As set out at Part 4.2.
- (g) REVIEW. That the arrangements be reviewed at six weeks, or earlier if either party considers it necessary.
- (h) GRADUATED RETURN. If the Health Service prefers, I am willing to return on a graduated basis by agreement. I am not seeking to avoid any part of my role.

If any part of this proposal cannot be accommodated, I ask that the Health Service identify which part and its reasons, having regard to G03 clause 2.

3. CONTROLS I WOULD BE WILLING TO SEE PUT IN PLACE

I record these subject to two qualifications I do not waive: identifying hazards and determining controls is a function of the Health Service under Part 3.1 of the Work Health and Safety Regulation 2011, not of the worker; and the controls that are appropriate are ultimately a matter for clinical opinion, which question 1(d) of the Health Service's own request seeks.

- (a) A telephone call one hour before each rostered shift, to confirm I am attending; if unanswered or attendance is not confirmed, a second call; if still not confirmed, the on-call operator is contacted and cover arranged. The measure is contingent, not standing: on the shifts I attend — most of them — it costs one telephone call, and where I am not attending, that is established an hour ahead rather than at start time. An on-call arrangement has existed in the unit since 15 April 2024; this applies it, and would have addressed a substantial part of the attendance history recorded in the correspondence of 8 September 2025. It is a confirmation of attendance, not an assessment of my condition — no colleague needs to know anything about my health for it to work.
- (b) A defined escalation pathway I may initiate during a shift if my condition is acutely exacerbated — who assumes the board, how the shift is closed out — so the response is planned rather than improvised.
- (c) Continuation of the existing complaint-handling arrangement recorded in the Health Service's own letter: immediate escalation to the Client Liaison Officer and/or Manager. This requires no change to anything.
- (d) Continuation of the rostered pattern that has applied over the preceding twelve months. The Checklist records it as "the pattern Mr Shepherd has in fact worked and tolerated over the past twelve months without deterioration" and a "continuation of existing arrangement". The Health Service set that roster; where my condition was exacerbated I managed it myself, from my own sick and annual leave — which is why, on 3 July 2026, my paid sick leave was exhausted. I am not asking for the pattern to be reduced. I am asking that it continue, with the variability managed by a process rather than by the exhaustion of my entitlements. If it is now said the role requires something different, I ask what changed, and when.
- (e) Injury management correspondence directed through the Injury Management team, consistent with the Checklist's own recommendation.

The only conduct matter ever raised with me is attendance and lateness; the measures at (a) and

(b) are directed at precisely that. If any measure cannot be accommodated I ask that the Health

Service identify which and why, noting G03 clause 2: unjustifiable hardship is "tested against the whole organisation, not a division or unit", and "the onus is on Queensland Health, as the employer, to prove an adjustment is unreasonable".

4. THE STATUS OF THIS PROPOSAL — A REQUEST UNDER CLAUSE 10.3

So that the Health Service is not left to determine for itself what kind of document this is, I identify the provision the proposal is made under.

Clause 10.3.2 of the certified agreement provides that, in accordance with the Industrial Relations Act 2016, an employee "may ask the employer for a change in the way the employee works, including the employee's ordinary hours of work." Section 27(1)(c) of that Act puts the matter beyond doubt for a case such as this one: the request may be for "a change to the way the employee works, for example, the use of different equipment as a result of a disability, illness

or injury."

The measures at sections 2 and 3 are a change in the way I work, sought as a result of an illness. I have made that request formally, by a separate letter of today's date headed "Request for a change in the way I work", so that there is no question about what it is or when it was received. That letter sets the change out in the form clause 10.3.3 requires and states the reasons for it. Sections 2 and 3 of this document are the fuller explanation of the same proposal.

I ask that the two be read together, and that the request be dealt with under clauses 10.3.4 to 10.3.6. Nothing in this Part is intended to displace a discussion by agreement, which I would much prefer, and I remain willing to consider any alternative the Health Service or my practitioners consider more appropriate.

For completeness, the request under clause 10.3 is separate from, and does not answer, the matters at Parts 4, 8, 9, 10 and 11 of this document, which are not made under that clause.

5. MY SUBSTANTIVE POSITION

The letters of 31 July 2026 describe my employment as "Administration Officer (AO), Switchboard Operations, Corporate Services, Logan Hospital (LH), Metro South Health (MSH) on a Permanent Full-time basis 76 hours per fortnight."

I ask the Health Service to confirm:

- (a) that the position so described continues to be held for me, and that no permanent appointment to it has been made or is proposed to be made while I am held out of the workplace;
- (b) whether the shifts I would ordinarily work are being performed by another employee. I ask because it bears directly on Question 9. The Health Service has asked my doctor to advise whether I can safely return if it is "not able to accommodate the restrictions". If the duties I would perform are being performed by someone else, the work is available, and the premise of that question requires examination before it is put to anyone; and
- (c) what, if anything, has been communicated within the workplace about my absence.

I raise (c) because my absence has been the subject of enquiry among colleagues. I would prefer that whatever is said about it is accurate.

I raise these matters only to have the position confirmed in writing.

I confirm again that I am engaging with this process and want to return to work.

Yours sincerely,

Cory Lea Shepherd

AO3, Switchboard Services, Logan Hospital

0417 400 227 · coryshepherd1@hotmail.com